Autism Spectrum Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

Brian Campbell, Ph.D.

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1. What it is

Autism spectrum disorder (ASD) is a lifelong neurodevelopmental condition marked by differences in social communication and by restricted, repetitive patterns of behavior, interests, or sensory response, present from early childhood (American Psychiatric Association [APA], 2022). It ranges from people who need very substantial daily support to independent adults with careers and families.

How common: in the U.S., 1 in 36 eight-year-olds (27.6 per 1,000) had ASD in 2020, about 3.8 times more often in boys (Maenner et al., 2023). Worldwide, the median estimate is about 1 in 100, and about one-third of autistic people also have intellectual disability (Zeidan et al., 2022).

Co-occurring conditions are the norm: pooled rates were 28% for ADHD, 20% for anxiety disorders, 11% for depression, and 9% for OCD (Lai et al., 2019).

Counselor framing: autism itself is not something counseling "cures." Counselors most often help with co-occurring anxiety and depression, family stress, and social and life skills, and help churches welcome autistic people and their families.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis, which requires specialist assessment.

- **Social communication** (all three): difficulty with back-and-forth social and emotional exchange; with nonverbal communication such as eye contact, gestures, and facial expression; and with developing, keeping, and understanding relationships.
- **Restricted, repetitive patterns** (two or more): repetitive movements, speech, or use of objects; insistence on sameness and distress at small changes; intense, narrow interests; and over- or under-reaction to sensory input such as sound, light, texture, or pain.
- **Onset and impact:** present in early development (though it may not be noticed until demands exceed abilities), causing significant impairment, and not better explained by intellectual disability alone.
- **Specifiers:** with or without intellectual or language impairment, and a support level of 1 ("requiring support"), 2 ("substantial support"), or 3 ("very substantial support").

Screening toddlers: on the M-CHAT-R/F, toddlers who screened positive after the follow-up interview had a 47.5% chance of an autism diagnosis and a 94.6% chance of some developmental delay or concern (Robins et al., 2014). Refer positive screens for full evaluation.

Safety: among adults newly diagnosed with Asperger's syndrome (now part of ASD), 66% reported past suicidal thoughts and 35% suicide plans or attempts (Cassidy et al., 2014). Autistic people also died at a higher rate than matched controls (2.6% vs. 0.9% over the study period) (Hirvikoski et al., 2016). Ask about suicide directly and in concrete language.

When counselors must refer (general clinical consensus): refer for diagnostic evaluation by a specialist team, and for psychiatric care for suicidality, severe aggression or self-injury, or co-occurring psychiatric conditions.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in autism spectrum disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
M-CHAT-R/F (Modified Checklist for Autism in Toddlers)	Screening toddlers for autism, with a follow-up interview.	Parent-report, 20 items, plus follow-up	No (free for clinical use)	None beyond general clinical training	M-CHAT website
AQ / AQ-10 (Autism Spectrum Quotient)	Screening for autistic traits in adults, teens, and children.	Self- or parent-report, 50 items (10-item short form)	No (free for clinicians and research)	None beyond general clinical training	Autism Research Centre, Cambridge
APA Clinician-Rated Severity of Autism Spectrum and Social Communication Disorders	Rating the DSM-5 level of support needed.	Clinician-rated, brief	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
SCQ (Social Communication Questionnaire)	Screening for autism in anyone over age 4.	Parent-report, 40 yes/no items	Yes	★ Level C	WPS
SRS-2 (Social Responsiveness Scale, 2nd ed.)	Severity of social impairment, age 2½ through adulthood.	Parent, teacher, relative, or self-report, 65 items	Yes	★ Level C	WPS
CARS-2 (Childhood Autism Rating Scale, 2nd ed.)	Clinician rating of autism features, ages 2 and up.	Clinician-rated after observation and interview	Yes	★ Level C	WPS
ADOS-2 (Autism Diagnostic Observation Schedule)	The standard observational assessment for autism, 12 months to adulthood.	Structured clinician observation, 40 to 60 minutes	Yes	★ Level C plus specialized training	WPS

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: early developmental and behavioral interventions help young children, though the best-controlled evidence is modest. For the anxiety that so often accompanies autism, CBT adapted for autistic people works well.

Evidence overview (honest findings):

- **Early intervention:** across studies of young children, behavioral, developmental, and naturalistic developmental-behavioral interventions showed positive effects; limited to randomized trials, only developmental and naturalistic approaches held up, and with the strictest controls for bias no approach showed significant effects (Sandbank et al., 2020). Evidence for early intensive behavioral intervention (EIBI) was rated low to very low quality (Reichow et al., 2018).
- **CBT for anxiety:** across 8 studies (469 children), CBT for anxiety in autistic children had large effects on clinician- and parent-rated anxiety (d = 1.19 and 1.21) and a moderate effect on self-report (d = 0.68) (Sukhodolsky et al., 2013). In a trial of 167 children, CBT **adapted for autism** outperformed standard CBT and usual care (Wood et al., 2020).
- **CBT for other problems:** CBT for co-occurring mental health problems and for autism-related symptoms showed small-to-medium effects that were stronger on clinician and parent ratings than on self-report; definitive trials are still needed (Weston et al., 2016).

Adapting CBT for autistic clients (Wood et al., 2020; Weston et al., 2016):

- **Be concrete and visual:** written agendas, worksheets, scales, and literal language rather than metaphors.
- **Use special interests** to engage and to illustrate ideas, and involve parents or partners to help practice skills.
- **Identify and challenge** anxious and self-critical beliefs: "Something terrible will happen if the routine changes," "Everyone thinks I'm weird," "I'm broken." Pair with gradual exposure and sensory accommodations.

5. Religion, spirituality, and Christian integration: what the evidence shows

Faith and autistic people. In interviews with 20 young people with autism or intellectual disability, many described faith as important, with affirmation and acceptance of their disability in the light of their faith (Liu et al., 2014).

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Faith and parents. Among mothers of autistic children, religious beliefs and spirituality went with better well-being, while religious *activities* were linked to more negative outcomes (Ekas et al., 2009). Among 73 Christian mothers, higher spirituality went with seeing their child as a positive contribution to the family, which in turn went with less anxiety; mothers of children with more severe autism reported more negative interactions with church members (Ekas et al., 2019). In a 15-country study of 1,687 parents, a 6-day spiritual lessons program was followed by greater resilience though without a control group (Pandya, 2018).

Churches can do better. In a survey of 416 parents, most said their faith was important, but were generally not satisfied with the support their faith communities provided, and highly valued a welcoming attitude (Ault et al., 2013).

Intervention studies. We found **no trial of Christian or religiously integrated CBT for autistic people** (PubMed search, October 31, 2026).

Honest summary for teaching:

1. Early intervention and autism-adapted CBT are the evidence base. Faith integration supports them; it does not replace them.
2. For parents, inner spirituality and positive ways of seeing their child go with less anxiety; difficult experiences at church may explain why religious activities sometimes add stress.
3. No faith-integrated treatment trial exists. The clearest opportunity is helping churches become welcoming and supportive.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within adapted CBT and family support. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** With parents, listen for beliefs such as "God is punishing us," "Our child is a burden," or "We don't belong at church." With autistic clients, listen for "I'm broken," "God made a mistake with me," or "I can't please God because I can't act like others." To identify deeper false beliefs, see: Core Beliefs, Parental Messages, or use the Semantic Search Tool.
- **Challenge (clinical).** Test these beliefs against the evidence of the child's or person's strengths and contributions; positive ways of seeing the child are linked to less parental anxiety (Ekas et al., 2019). Use concrete, literal language with autistic clients (Wood et al., 2020). **Possible cognitive errors** to watch for in parents and in autistic clients, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
  - **Global labeling** — one mistake becomes who I am. *Feeds:* shame (in autistic clients). *Sounds like:* "I'm broken."
  - **Comparisons** — measuring myself against others. *Feeds:* low self-worth and withdrawal. *Sounds like:* "Everyone else knows how to act at church."
  - **Mind reading** — assuming I know what others think. *Feeds:* social anxiety. *Sounds like:* "They all think I'm weird."
  - **All-or-nothing thinking** — no middle ground. *Feeds:* distress when plans change. *Sounds like:* "If the plan changes, the whole day is ruined."
  - **Personalization** — taking blame for things outside my control. *Feeds:* parental guilt. *Sounds like:* "God is punishing us."
  - **Fortune telling** — predicting a bad outcome. *Feeds:* parental hopelessness. *Sounds like:* "She'll never have a good life."
  - **Disqualifying the positive** — deciding good things don't count. *Feeds:* parental discouragement. *Sounds like:* "Our child is a burden."
- **Questions to dispute the belief.** Have the parent or client choose questions from the Questioning False Beliefs form that "speak to" the belief. With autistic clients, ask one concrete question at a time. To see how, watch Dr. Campbell's video Integrated Christian Counseling: Questioning Your Beliefs. Selected questions that fit the errors above:
  - "Doesn't everyone have good points and bad points?" and "Does God still love me despite my faults or limitations?" *For:* global labeling and "God made a mistake with me".
  - "Am I being too hard on myself?" *For:* comparisons.
  - "What's the actual evidence for my belief?" *For:* mind reading.
  - "Am I keeping things in perspective?" *For:* all-or-nothing thinking.
  - "Am I really the cause of the problem, or did someone just try to convince me that I am?" *For:* parental personalization.

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    - "Can I really predict the future?" *For:* fortune telling.
    - "Have I overlooked some of the gifts that God has given me?" *For:* disqualifying the positive.
- **Replace (pastoral).** Offer Scripture of God's design and the value of every member of the body:
    - "For You formed my inmost being; You knit me together in my mother's womb. I praise You, for I am fearfully and wonderfully made. Marvelous are Your works, and I know this very well." (Psalm 139:13–14)
    - "On the contrary, the parts of the body that seem to be weaker are indispensable." (1 Corinthians 12:22). Every member is needed in the church.
    - "He tends His flock like a shepherd; He gathers the lambs in His arms and carries them close to His heart. He gently leads the nursing ewes." (Isaiah 40:11). God's care for children and weary parents.
    - "Carry one another's burdens, and in this way you will fulfill the law of Christ." (Galatians 6:2)
- **Faith community (clinical).** Help churches offer predictable routines, quiet spaces, sensory accommodations, a buddy or volunteer, and respite for parents (Ault et al., 2013).
- **Ministers:** welcome the family, ask parents what helps, and refer for evaluation or counseling; never suggest autism results from sin or lack of faith.

7. Quick classroom takeaways

- ASD involves differences in social communication plus restricted, repetitive patterns from early childhood. About 1 in 36 U.S. children are diagnosed; anxiety, ADHD, and depression commonly co-occur. Ask about suicide plainly.
- Early developmental interventions help young children; CBT adapted for autism treats anxiety well. Be concrete, visual, and involve families.
- Faith matters to many autistic people and their parents. Replace "broken" and "burden" with the truth that God formed each person and that every member of the body is indispensable.
- Free screeners (M-CHAT-R/F, AQ) flag concern; formal diagnosis typically uses Level C tools such as the ADOS-2, which require specialized training.

Sources note

All 17 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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